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Clinical Study Title:	**KISS Study (Keep bIfurcation Single stenting Simple)**
Sponsor:	**CERIC** (CEnter for European Research Initiatives in Cardiovascular medicine)
Clinical Investigation Plan **Version #:** **Version date:**	 V1.0 07 September 2020

Sponsor's Representative approval	
Name:	DR PHILIP URBAN
Function:	General Manager
Date:	09-Sep-20
Signature:	DocuSigned by: Philip Urban 2547F9994B82441...

Version 1.0, dated September 07, 2020

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2 Corelab Plan: Objective and context

The purpose of the Corelab Plan is to provide:

- an overall presentation of the corelab activities related to the project,
- the description of the organization and analyses to be performed,
- the schedule for providing the Corelab Report.

3 Management of the Corelab Plan

The Clinical Projet Leader in collaboration with the Corelab Specialist is responsible for issuing and updating the Corelab Plan throughout the study in case of any change of the Clinical Investigation Plan that could have an impact on the content of the Corelab Plan.

This Corelab Plan is reviewed by the Medical Corelab Manager and approved by:

- the CERC Medical Director,
- the study Sponsor.

4 Project Specificities

4.1 Project main features

4.1.1 Study design

KISS study is a multi-centre, prospective, randomized (1:1), parallel, two-arm, non-inferiority performed in up to 6 countries where Medtronic Resolute Onyx stent is CE approved.

4.1.2 Objectives and Endpoint(s) of the study

KISS study aims to evaluate the non-inferiority of no side branch (SB) intervention versus side branch ballooning, in the setting of single-stent bifurcation technique with systematic proximal optimization technique (POT).

- **Primary Endpoint(s):**

Rate of peri-procedural Myocardial Infarction (MI) at discharge or 48h post-procedure (whichever comes first) as judged by clinical evaluation in association with day-1 troponin evaluation using the ARC-2 definition.

- **Secondary Endpoint(s) related to corelab analysis:**

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Predefined Secondary Procedural Endpoints:

- Technical success: <20% residual stenosis in main branch (MB) stent and patent side branch at end of procedure as reported by corelab,
- Acute gain in main and side branches as measured with a dedicated quantitative coronary analysis (QCA) software,
- Rates of crossover for side branch strategy,
- Comparison SB ballooning vs Kissing Balloon Technique (KBT) in SB intervention arm

Clinical endpoints at **1 month** and **12 months:**

- Target Lesion Failure (TLF), composite of:

  - Target vessel Myocardial Infarction

  - Clinically indicated Target lesion revascularization (ci-TLR)
- Main Branch & Side Branch TLR
- Definite or probable Stent Thrombosis (ARC-2)

The corelab will perform QCA on revascularisation events.

4.1.3 Project requirements

Up to 20 sites over up to 6 European countries. 596 consecutive patients are planned to be enrolled

4.1.4 Project schedule, i.e. period and planning,

The Project schedule is detailed in the CIP (see Annex 1):
Estimated duration of the study: 35 months.

5 Corelab process overview

This section is applicable for angiography Qualitative and Quantitative Coronary Analyses.

5.1 Site Corelab Package

At the study set-up, the CPL and Corelab Specialist:

1. prepare the following documents for the site(s):
   - Angiographic Instructions to the site (Annex 2)

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- Technician's Worksheet Form Angiography (TWS) (Annex 3)

2. and sends them to the Medical Corelab Manager, and to the CERC Medical Director for review and approval.

Once approved, the Clinical Trial Assistant (CTA) files the Instructions to the site and the Technician's Worksheet in each Investigator Regulatory File and in the Corelab section of the CERC electronic Trial Master File (eTMF).

These documents are presented to the site staff during the Site Initiation Visit.

5.2 Corelab database

The CPL and the Corelab Specialist prepare the data collection forms. These forms will be used to document each analysis.

The Corelab Specialist:

(1) prepares all the Corelab analysis forms:

- Angiography CRF Form Pre, Post procedure (Annex 4)
- Angiography CRF Form Revascularisation Event (Annex 5)
- Angiography Checking/Feedback Form N°1 and N°2 (Annex 6) and notification form.

(2) And sends them for review to the Medical Corelab Manager.

After validation by the CERC Medical Director and the Sponsor the Corelab Specialist sends them to the Data Manager who will design the Corelab Database according to the applicable data management procedures (DM-SOP-010).

5.3 Image and documents (upload, reception and first assessment)

Anonymized images, in DICOM format only, will be uploaded by the sites through the *Decidemedical platform.* The maximum recommended time for the upload of the films is 10 business days from procedure.

Upon receipt of the films and related documents sent by the site, the **Corelab Assistant:**

- performs a first quality check within **5 working days** using Angiography Feedback form N°1 (Annex 6). This is transmitted to the study site directly via the EDC or by email.
- completes the "Corelab imaging documents tracker"
- files copies of the anonymized images and documentation in the e-TMF.

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- forwards the imaging documents (images, technician worksheet, procedure report) to the Corelab Specialist for analysis

5.4 Analysis process

The Corelab Specialist performs:

1. corelab analyses within **45 calendar days** from the reception of Corelab images and documentation according to methodology as described in:
   - Corelab Standard Angiographic methodology (CL-SOP-001d) (Annex 7)
   - Angiographic bifurcation methodology (CL-SOP-001e) (Annex 8)
   - Corelab for Event Methodology (CL-SOP-001i) (Annex 9)
2. the data entry into the Corelab EDC.
3. a second quality check using the Angiography feedback form N°2 (Annex 6) and transmits it directly to the site via the EDC or by email.

For revascularization events the analyses are performed within 5 working days.

All corelab data are kept electronically in the e-TMF with restricted access to the Corelab Specialist, Medical Corelab Manager and the Data Manager.

5.5 Analysis review and Approval

The Corelab Medical Expert assesses the consistency of the results and approves (dates and signs) the analysis.

This review should be performed within **45 calendar days** of the analysis.

If appropriate, re-analysis may be performed upon request from the Corelab Medical Expert.

6 Angiography Analysis methodology

Qualitative and quantitative angiographic analyses will be performed for Pre and Post procedure and for revascularisations events.

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6.1 Qualitative Analysis and sceening

6.1.1 Coronary Segment

For each analyzed lesion, the Corelab Specialist assigns a Core Cass Number determining the segment stenosis location as listed in Table 1 and represented in Figure 1.

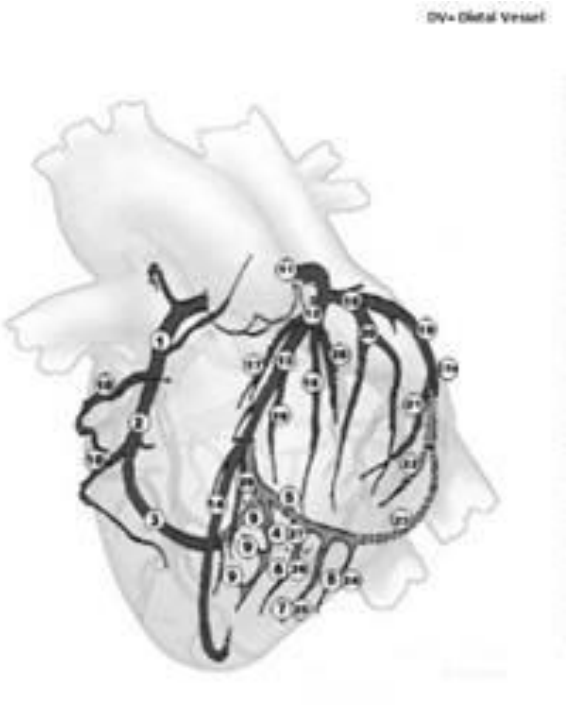

Figure 1: Core Cass Numbers (inside the circles) representing different coronary segment locations

Table 1: The Core Cass numbers attributed to different coronary segment locations

Segment	CASS Number	Segment	CASS Number
RCA, proximal	1	Diagonal 2	16
RCA, mid	2	Diagonal 3	29
RCA, distal	3	1st Septal	17
PDA	4	Left Circumflex, proximal	18
PLSA	5	Left Circumflex, mid	19
RPL1	6	Left Circumflex, distal	19a
RPL2	7	Obtuse Marginal 1	20
RPL3	8	Obtuse Marginal 2	21
Righ Septal	9	Obtuse Marginal 3	22
Right Ventricular	10	Left Atrioventricular	23
Left Main	11	LPL1	24
LAD, proximal	12	LPL2	25
LAD, mid	13	LPL3	26

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Segment	CASS Number	Segment	CASS Number
LAD, distal	14	Left PDA	27
Diagonal 1	15	Ramus (optional)	28

A unique segment identification system is used for identifying the coronary segment treatment site as described below:

- A prefix '1' is added to the CASS segment to designate saphenous vein grafts, coded to the distal anastomotic insertion of the conduit.
  - ➔ Example: Vein graft to the PDA, CASS#=104
- A prefix '2' is affixed to designate left internal mammary artery (LIMA) grafts.
  - ➔ Example: LIMA's usually tie into the LAD, so LIMA to the distal LAD, CASS# = 214
- A prefix of '3' is affixed to designate right internal mammary artery (RIMA) grafts.

6.1.2 Qualitative Morphology Assessment:

Pre-procedural and post-procedural qualitative morphology are assessed.

The following angiographic criteria will be recorded for proximal main (PM), distal main (DM) and side branch (SB)

Baseline:

Criteria	
Angle	Pre-randomisation SB ballooning
Medina Bifurcation	Calcification
Segment Treated	TIMI Flow Pre
Geographic Miss	Thrombus
Contour Corrected %	Aneurysm
Length obstruction	Film quality

- **Post Procedure:**

Criteria	
TIMI Flow Post	Distal Embolisation
Angulation Post	Perforation
Thrombus	**Technique:**
Spasm	POT (Post main branch treatment)
Abrupt closure	Side branch ballooning
No Reflow	Kissing Balloon
Dissections/staining	Final POT
If Dissections: length and classification	Overlap / Gap (length) and localisation
Residual stenosis	

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- **Revascularisation /Event Procedure:**

Criteria	
TIMI Flow	Pattern ISR
Medina Bifurcation	Film quality
Angulation	Thrombus
Length obstruction	Dissections/staining and length and classification
Contour corrected %	

6.2 Quantitative angiographic Analysis (QCA)

The software used for the QCA in the study is CAAS 7.3 (Pie Medical). The selected software will be used for the duration of the study.

6.2.1 Image Calibration

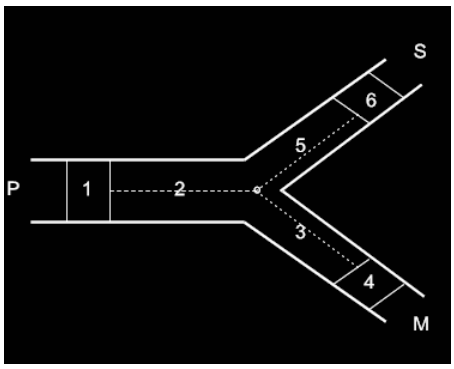
Calibration is performed using the contrast-filled catheter.

6.2.2 Segment Selection

Normal and diseased arterial segments are selected for analysis.
A computer-assisted edge detection algorithm is applied to obtain quantitative coronary dimensions.
A 5mm segment of 'normal' reference vessel is selected proximal and distal to the stenosis and averaged for determination of 'user-defined' reference diameters.
Normal and minimal lesion diameters are calculated from the single most severe and tightest projection.

6.2.2.1 Segmental analysis

Dedicated bifurcation softwares allow a segmental analysis (stented, edges, ostial…) and provide a reporting system with 6 segments.



1. Proximal edge (5mm)
2. Proximal main stent
3. Distal main stent
4. Distal edge main (5mm)
5. Side branch stent
6. Distal edge side (5mm)

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6.2.2.2 Pre, Post and Event Procedure measurement

Analysis at the 6 segments:

Procedure measurements	
Segment length	mm
Minimum Lumen Diameter diameter - MLD	mm
Diameter- stenosis	%
Interpolated Reference - diameter	mm
Maximum diameter in segment	mm
Mean diameter in segment	mm

For revascularizations, QCA measurement is performed for TLR /TVR.

For TVR non TLR only the % diameter stenosis is measured. QCA diameter stenosis of > 70% based on the average of multiple views or if not feasible based on the 'worst view' are considered clinically indicated revascularisations.

7 Corelab Data management and statistical analyses

After validation by the Medical Expert, the Data Manager performs all corresponding data entry checks as well as data transfer checks according to "Data Entry and management of external data" (DM-SOP-004) procedure.

At the end of the study the Data Manager reviews and cleans the database.

If contracted the Data Manager sends the SAS exports to the Biostatistician, who will perform the statistical report according to the study Statistical Analysis Plan.

8 Corelab Report

As contractually agreed, once all Corelab analyses are performed, the Corelab Specialist, in collaboration with the Data Manager, Medical Corelab manager and the Biostatistician, issues a Corelab Report within 30 calendar days.

This Corelab Report is then reviewed and validated by the Medical Director and/or the Medical Corelab Manager within 2 weeks. The Sponsor and/or the PI will also validate the report.

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9 Analysis documentation and archiving

All the analysis, database and scanned Corelab documents are filed in the Corelab hard drive.

All the paper documents, CD, DVD and the data collection forms filled in by the Corelab Specialist, are archived in the Trial Master File. A folder is created for each patient.

At the end of the trial the CERC TMF will be transferred to the Sponsor.

10 Terminology / abbreviations

ARC-2	Academic Research Consortium
CCS	Canadian Cardiovascular Society
CEC	Clinical Events Committee
CMS	Cardiovascular Angiographic Analysis System (CAAS-II)
CRF	Case Repot Form
CSV	Microsoft Excel Comma Separated Values File
CTA	Clinical Trial Assistant
DICOM	Digital Imaging and Communication in Medicine
DMV	Distal Main Branch
EDC	Electronic Data Capture
e-TMF	e-Trial Master File
ISR	Intra-Stent Restenosis
IVUS	Intravascular ultrasound
MI	Myocardial Infaction
MLD	Minimal Lumen Diameter
PCI	Percutaneous Coronary Intervention
PMV	Proximal Main Vessel
POT	Proximal Optimization Technique
QCA	Quantitative Angiographic Analysis
SB	Side Branch
TIMI	Timi Flow
TLF	Target Lesion Failure
(ci-)TLR	(clinically driven-) Target Lesion Revascularization
TVF	Target Vessel Failure
TVR	Target Vessel Revascularisation

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TWS	Technician Work Sheet

11 Appendices and other related documents

Annex 1: KISS Clinical Investigation Plan (applicable version)

Annex 2: Angiographic Instructions to the site

Annex 3: Technician's Worksheet Form Angiography

Annex 4: CRF Angiography analysis Form Pre, Post

Annex 5: CRF Angiography analysis Form Event

Annex 6: Angiography Feedback Form N°1 and N°2 (CL-SOP-001c) and notification form

Annex 7: Standard Angiographic Methodology (CL-SOP-001d)

Annex 8: Angiographic Bifurcation Methodology (CL-SOP-001e)

Annex 9: Corelab for Event Methodology (CL-SOP-001i)

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